INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Lapatinib + capecitabine

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00217
Version 7

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

Treatment stages

01 **Days 1–14 · both tablets**
21-day cycle

02 **Days 15–21 · lapatinib continues**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, dehydration, jaundice, unusual bleeding or sudden illness promptly. Do not wait for the next planned test.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

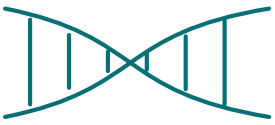
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each medicine does

Control the cancer and symptoms, with repeated review of benefit and tolerance.

ORAL HER2 / EGFR KINASE INHIBITOR

Lapatinib



Blocks signals inside cells

Lapatinib inhibits HER2 and EGFR signalling inside cancer cells. It is used with capecitabine in selected previously treated HER2-positive advanced breast cancer.

Target in the cell → Blocks signals inside cells → Cancer growth disrupted

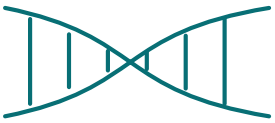
What to watch for

Report new or worsening symptoms promptly and follow the agreed contact plan.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

The body converts capecitabine into fluorouracil, which disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA processes needed for cell growth. Tablets are active chemotherapy; taking them at home still requires a clear monitoring and interruption plan.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This all-tablet combination may be considered when HER2-positive advanced cancer has progressed after previous treatment including an anthracycline, a taxane and trastuzumab in the metastatic setting. It blocks growth signals and uses chemotherapy through a different mechanism. Earlier medicines, cancer sites, kidney and liver health, tolerance and available alternatives affect the choice.

What could change the choice?

Ask which cancer features, earlier treatments and health factors led to this recommendation, which alternatives were considered and when the decision will be reviewed.

Your treatment and the next step

Schedule

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

After this phase

Continue while treatment is helping and tolerated. If a component stops, confirm the exact medicines, tests and supportive prescription for the new phase. Maintenance remains active treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Treatment stages

01 **Days 1–14 · both tablets**
21-day cycle

02 **Days 15–21 · lapatinib continues**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

Days 1–14 · both tablets

21-day cycle

Lapatinib once daily; capecitabine twice daily on the prescription.

21-DAY CYCLE

Days 15–21 · lapatinib continues

21-day cycle

Capecitabine pauses for seven days; lapatinib continues daily unless the team instructs otherwise.

Relative sequence, not equal lengths or booked dates. Your team confirms actual appointments and the active medicines.

BASELINE → TREATMENT → FOLLOW-UP

Tests and milestones through treatment

Columns show sequence, not equal durations or appointments. A test is only complete when performed and reviewed. Echo timing and cancer-imaging timing are different. Symptoms can bring any check forward.

Check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Blood tests	Baseline full blood count (FBC), renal and liver profile; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result; potassium / magnesium	Before each 21-day cycle; assess symptoms and oral dosing	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
Echo / cardiac assessment	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / multigated acquisition (heart pumping) scan (MUGA) due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
electrocardiogram (ECG) / QT	Baseline ECG; correct K / Mg	Repeat 1–2 weeks after starting (cycle 1 Days 8–15)	Additional ECG if risk, interacting drugs or symptoms	Review QT-risk medicines and electrolytes	Individual ECG plan; early ECG is not an echo
Cancer imaging / markers	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan
Surgery / radiotherapy	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects

Check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Endocrine / bone health	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	dual-energy X-ray absorptiometry (bone density) (DEXA) when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00217 v7 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Lapatinib + capecitabine: the whole pathway

HER2-positive advanced breast cancer · previously treated

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Cancer medicines NCCP	Confirm diagnosis and exact regimen	See the dose-day map: lapatinib daily; capecitabine Days 1–14	Echo due; treatment review	Review active medicines and benefit / tolerance	Review benefit / tolerance and next plan
Surgery / radiotherapy METASTATIC	Agree indication and sequence	No automatic concurrent radiation	Coordinate with both teams	Local treatment as indicated	Review recovery and late effects
Blood tests NCCP	Baseline FBC, renal and liver profile; DPD result; potassium / magnesium	Before each 21-day cycle; assess symptoms and oral dosing	Continue the required pre-dose checks	Use the selected active regimen's blood-test schedule	Investigate symptoms / persistent abnormalities
ECG / QT SMPC	Baseline ECG; correct K / Mg	Repeat 1–2 weeks after starting (cycle 1 Days 8–15)	Additional ECG if risk, interacting drugs or symptoms	Review QT-risk medicines and electrolytes	Individual ECG plan; early ECG is not an echo

Care / check	Before starting	First cycles	Around week 12	Continuing / next phase	Longer-term plan
Echo / cardiac checks NCCP · ESC	Baseline cardiac assessment	Earlier for symptoms / concern	Echo / MUGA due around 12 weeks	Continue every 12 weeks; keep the original clock	Agree follow-up for the active medicine and cardiac risk
Scans / tumour markers METASTATIC · MARKERS	Baseline disease assessment	Clinical response; imaging if concern	Response review as planned; not tied to echo	Reassess disease and treatment plan	Imaging generally every 2–4 months; individual plan
Endocrine / DEXA BONE	Check hormone receptors / fracture risk	Plan endocrine therapy if indicated	DEXA when relevant to bone-risk plan	Endocrine timing by the treating team	Individual DEXA / bone follow-up

What each check is for

1

Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

Check safety before the next dose.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

2

Heart tests

NCCP requires baseline left ventricular ejection fraction (heart pumping measurement) (LVEF) assessment in its cardiac precautions and echo/MUGA every 12 weeks (around cycle 5 if on time), or earlier for concern. EMA Tyverb specifies an ECG with QT before starting and again 1–2 weeks after starting, around Days 8–15 of cycle 1. Correct low potassium or magnesium before treatment and arrange extra ECG checks for interacting medicines, symptoms or QT risk. This early ECG is separate from the 12-week echo.

Monitor pumping function and relevant rhythm risk.

Ask / act: Agree the booking owner, result reviewer and action if overdue.

3



Cancer imaging

Record baseline disease sites. Response imaging is generally reviewed every 2–4 months under international guidance, adapted to symptoms, disease pace and treatment. The team selects the scan; this is not an automatic computed tomography (CT) chest/abdomen/pelvis order for every patient. Tumour markers may support assessment in selected cases, but do not alone decide progression or a treatment change.

Assess response or follow-up needs.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

4

Bone health

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

Assess fracture risk where relevant.

Ask / **act:** Agree the booking owner, result reviewer and action if overdue.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical tablet plan

Both anticancer medicines are taken by mouth. This regimen itself requires no infusion pump or port.



STEP 1

Before starting

Confirm DPD result, blood tests, cardiac assessment, medicines review and the two tablet schedules.



STEP 2

Collect and check the prescription

Confirm tablet strengths, doses, Days 1–14 for capecitabine and continuous lapatinib. Keep in the original packaging and out of reach of children.

STEP 3

Food and medicine timing

Take lapatinib at least one hour before or after food, consistently in the same relation to meals. Take capecitabine with food or within 30 minutes after eating. Swallow tablets whole. A pharmacist can help arrange the different food timings.

STEP 4

Before each new cycle

Keep blood and clinical-review appointments even though treatment is at home. Bring your medicine list and tell the team about missed or held doses.

Interactions

Check all prescription, over-the-counter and herbal medicines with the oncology pharmacist. Acid-reducing medicines can affect lapatinib absorption: NCCP specifies separation by at least two hours, while the product information cautions against acid-suppressing combinations. Do not start or stop a necessary acid medicine yourself; agree a plan. Avoid grapefruit and report any recent brivudine use.

Missed or vomited tablets

Do not make up or double a dose. Follow the written instructions and call if unsure.

If you already have a port

Ask the line team whether it is still needed and how it should be maintained or removed. Removal is a separate planned procedure.

When scans, heart checks and bloods happen

Agree who books each test, who reviews the result and what happens if a check is overdue.

Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

Heart ultrasound and ECG

NCCP requires baseline LVEF assessment in its cardiac precautions and echo/MUGA every 12 weeks (around cycle 5 if on time), or earlier for concern. EMA Tyverb specifies an ECG with QT before starting and again 1–2 weeks after starting, around Days 8–15 of cycle 1. Correct low potassium or magnesium before treatment and arrange extra ECG checks for interacting medicines, symptoms or QT risk. This early ECG is separate from the 12-week echo.

Scans and tumour markers

Record baseline disease sites. Response imaging is generally reviewed every 2–4 months under international guidance, adapted to symptoms, disease pace and treatment. The team selects the scan; this is not an automatic CT chest/abdomen/pelvis order for every patient. Tumour markers may support assessment in selected cases, but do not alone decide progression or a treatment change.

Bone health and endocrine treatment

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

How surgery and radiotherapy fit

Surgery or radiotherapy may be used for particular symptoms or disease sites. There is no routine surgery-then-radiation sequence for this advanced-cancer regimen. The oncology and radiation teams agree any treatment pause or overlap.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
ECG / QT · cycle 1 only	 **Days 8–15** 1–2 weeks after starting
Lapatinib · every day	 **Days 1–21** Once daily
Capecitabine · Days 1–14	 **Days 1–14** Twice daily
No capecitabine · Days 15–21	 **Days 15–21** Lapatinib continues
Diarrhoea / hydration	 **Days 1–21** Report early
Hands / feet	 **Days 1–21** Pain affecting tasks: contact the team

Planned treatment: Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

Next phase: Continue while treatment is helping and tolerated. If a component stops, confirm the exact medicines, tests and supportive prescription for the new phase. Maintenance remains active treatment.

Heart checks: NCCP requires baseline LVEF assessment in its cardiac precautions and echo/MUGA every 12 weeks (around cycle 5 if on time), or earlier for concern. EMA Tyverb specifies an ECG with QT before starting and again 1–2 weeks after starting, around Days 8–15 of cycle 1. Correct low potassium or magnesium before treatment and arrange extra ECG checks for interacting medicines, symptoms or QT risk. This early ECG is separate from the 12-week echo.

Why these bands stay broad: Treatment markers show the planned schedule. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00217 v7 · official regimen ↗

There are no guaranteed "safe days". Serious symptoms need assessment on any day, even if the chart shows a usual recovery period.

Between-treatment notes

Day 1–21: The current treatment cycle

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week. Your team confirms treatment dates and checks. Serious symptoms need advice on any day.

Support around the medicines

1 **A clear tablet schedule**

2 **Nausea and bowel plan**

These oral agents are classified as minimal-to-low emetic risk. Agree an individual symptom plan; a repeated intravenous (IV) chemotherapy antiemetic course is not automatically required.

3 **Early symptoms matter**

Call promptly for diarrhoea, dehydration, painful hands or feet, mouth ulcers or poor intake. New chest pain needs urgent assessment.

4 **Before restarting a cycle**

Review bloods, kidney and liver function, symptoms and the agreed doses. Do not restart tablets after a treatment hold without the team's instructions.

Two tablets, two different schedules

Medicines review

Missed dose or vomiting

Do not double up or replace a vomited lapatinib dose. Follow the written missed-dose plan and contact the team if unsure. Significant diarrhoea, painful hands/feet, mouth ulcers or inability to drink may require tablets to be paused: call promptly for instructions.

Before first capecitabine

DPD testing must be reviewed before starting capecitabine unless already tested. The result can change the plan. Testing does not remove the need to report early or severe side effects.

Questions worth asking

Who arranges my tests?

Ask the team who books each test, who checks the result and who to contact if an appointment is overdue. This map does not book appointments.

What if I become unwell between visits?

Use the urgent contact plan for sudden illness, fever, chest symptoms, severe diarrhoea or poor fluid intake. Do not wait for a scheduled blood test or scan.

Does the rest week apply to both medicines?

No. Days 15–21 are a capecitabine break; lapatinib continues each day unless your team tells you to hold it.

Try explaining the plan back: Which medicines are active now, which days do you receive or take them, and who is booking and reviewing your next tests?

Lapatinib

- Diarrhoea and dehydration
- Rash
- Liver injury
- Heart pumping-function or rhythm changes
- Rare lung inflammation

Capecitabine

- Hand-foot syndrome
- Mouth ulcers / reduced intake
- Low blood counts and infection
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-related toxicity
- Renal impairment and medicine interactions

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1–14 · lapatinib and capecitabine

Active medicines: Lapatinib + Capecitabine

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine premedication is specified. Choose individual rescue antiemetics only after QT, liver, kidney and interaction review.
- Agree an early diarrhoea plan. NCCP example: loperamide 4 mg initially then 2 mg after each loose stool, maximum 16 mg/day or local policy. Severe diarrhoea or dehydration requires clinical assessment and dose-hold review.
- Capecitabine food timing differs from lapatinib. Pharmacy review is required for acid-suppressing medicines; separation alone does not reliably resolve a PPI interaction.
- Skin / hand-foot care, dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) status, current renal function and current toxicity must be reviewed before a new cycle.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 15–21 · lapatinib continues

Active medicines: Lapatinib

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- This is the planned capecitabine rest week, not a separate maintenance regimen.
- Continue the prescribed lapatinib unless held by the treating team. Do not replace missed or vomited doses.
- Maintain diarrhoea support, interaction review and urgent-contact advice. Do not carry a cytotoxic intravenous (IV) antiemetic course forward by default.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Lapatinib + capecitabine

Lapatinib once daily on Days 1–21, continuously. Capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Repeat every 21 days while beneficial and tolerated. The capecitabine rest week is not a lapatinib rest week.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Diarrhoea / hydration

Both medicines

Report diarrhoea early, especially dizziness, poor intake or reduced urine.

2 Hands / feet

Capecitabine

Painful redness or peeling affecting activity needs prompt advice.

3 Liver / skin colour

Especially lapatinib

Report yellow skin or eyes, dark urine or severe itching.

4 Heart / chest

Both medicines

New chest pain, breathlessness, palpitations or swelling needs urgent assessment.

5 Mouth / appetite

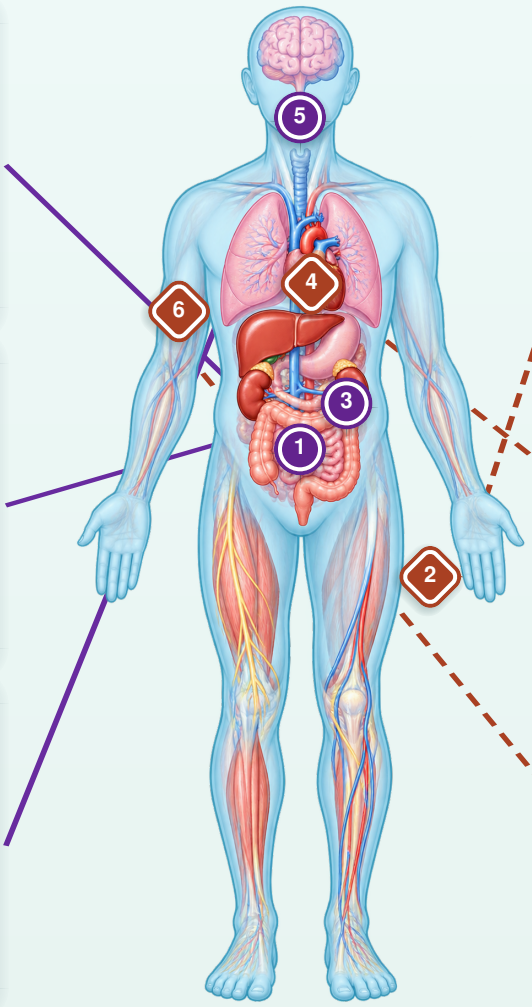
Capecitabine

Ulcers or difficulty eating and drinking need advice.

6 Rash / skin

Lapatinib

Report widespread rash, blistering or fever promptly.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The map shows selected effects of the medicines named here. Report persistent effects from earlier treatment too.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00217 v7 · official regimen**
   https://healthservice.hse.ie/documents/6559/217_Lapatinib_and_Capecitabine_therapy.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **EMA Tyverb product information**
   https://www.ema.europa.eu/en/documents/product-information/tyverb-epar-product-information_en.pdf
7. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **eviQ · implanted port patient information**
    https://www.eviq.org.au/p/507
11. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
12. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
13. **EMA Herceptin product information**
    https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
14. **EMA Perjeta product information**
    https://www.ema.europa.eu/en/documents/product-information/perjeta-epar-product-information_en.pdf